

1. Polyhydramnios, Oligohydramnios & Anemia in Pregnancy (Q1 – 30)

1. Definition: Polyhydramnios is diagnosed when:

- A) AFI $\leq$ 5 cm
- B) AFI $\geq$ 18 cm
- C) AFI $\geq$ 24 cm
- D) Single deepest pocket $<$ 2 cm

Answer: C) AFI $\geq$ 24 cm

2. Which maternal condition is most strongly associated with polyhydramnios?

- A) Chronic hypertension
- B) Gestational diabetes
- C) Asthma
- D) Hypothyroidism

Answer: B) Gestational diabetes

3. A common fetal cause of polyhydramnios is:

- A) Renal agenesis
- B) Esophageal atresia
- C) Bilateral renal dysplasia
- D) Intrauterine growth restriction

Answer: B) Esophageal atresia

4. Which medication is used with caution in polyhydramnios management due to risk of fetal ductus arteriosus closure?

- A) Nifedipine
- B) Indomethacin
- C) Labetalol
- D) Methyldopa

Answer: B) Indomethacin

5. Oligohydramnios is diagnosed when:

- A) AFI $\geq$ 24 cm
- B) AFI $\leq$ 5 cm
- C) Single deepest pocket $\geq$ 8 cm
- D) AFI between 5 – 24 cm

Answer: B) AFI $\leq$ 5 cm

6. A common cause of oligohydramnios is:

- A) Maternal diabetes
- B) Placental insufficiency
- C) Twin-twin transfusion syndrome
- D) Fetal anemia

Answer: B) Placental insufficiency

7. Which fetal anomaly is most linked to oligohydramnios?

- A) Anencephaly
- B) Duodenal atresia
- C) Bilateral renal agenesis
- D) Hydrocephalus

Answer: C) Bilateral renal agenesis

8. Oligohydramnios increases the risk of:

- A) Fetal macrosomia
- B) Cord compression
- C) Polyhydramnios later
- D) Maternal hypertension

Answer: B) Cord compression

9. According to WHO, anemia in pregnancy is defined as Hb:

- A) < 10 g/dL
- B) < 11 g/dL
- C) < 12 g/dL
- D) < 13 g/dL

Answer: B) < 11 g/dL

10. The most common cause of anemia in pregnancy is:

- A) Folate deficiency
- B) Vitamin B12 deficiency
- C) Iron deficiency
- D) Hemoglobinopathy

Answer: C) Iron deficiency

11. Severe anemia in pregnancy is classified as Hb:

- A) < 9 g/dL
- B) < 8 g/dL
- C) < 7 g/dL
- D) < 6 g/dL

Answer: C) < 7 g/dL

12. A maternal complication of severe anemia is:

- A) Polyhydramnios
- B) Preterm labour
- C) Gestational diabetes
- D) Oligohydramnios

Answer: B) Preterm labour

13. First-line treatment for iron-deficiency anemia in pregnancy is:

- A) Blood transfusion
- B) IV iron infusion
- C) Oral iron therapy
- D) Intramuscular iron

Answer: C) Oral iron therapy

14. A fetal complication of maternal anemia is:

- A) Macrosomia
- B) Intrauterine growth restriction
- C) Polyhydramnios
- D) Congenital heart disease

Answer: B) Intrauterine growth restriction

15. In malaria-endemic regions like Ghana, prevention of anemia includes:

- A) Avoiding iron supplements
- B) Deworming after first trimester
- C) High-protein diet only
- D) Vitamin C mega-dosing

Answer: B) Deworming after first trimester

16. Polyhydramnios increases maternal risk of:

- A) Postpartum hemorrhage
- B) Pre-eclampsia
- C) Chronic hypertension
- D) Gestational diabetes

Answer: A) Postpartum hemorrhage

17. Oligohydramnios in the second trimester suggests:

- A) Fetal gastrointestinal anomalies
- B) Fetal renal anomalies
- C) Maternal diabetes
- D) Twin pregnancy

Answer: B) Fetal renal anomalies

18. Thick meconium in labour is most concerning with:

- A) Polyhydramnios
- B) Oligohydramnios
- C) Maternal anemia
- D) Gestational hypertension

Answer: B) Oligohydramnios

19. For suspected hemoglobinopathy, the diagnostic test is:

- A) Liver function test

B) Hb electrophoresis

C) Renal ultrasound

D) Coagulation profile

Answer: B) Hb electrophoresis

20. Mild asymptomatic polyhydramnios is managed by:

A) Immediate induction

B) Serial ultrasound monitoring

C) Therapeutic amniocentesis

D) Cesarean section at 37 weeks

Answer: B) Serial ultrasound monitoring

21. Which is NOT a cause of polyhydramnios?

A) Maternal diabetes

B) Twin-twin transfusion syndrome

C) Fetal renal agenesis

D) Esophageal atresia

Answer: C) Fetal renal agenesis

22. Angular cheilitis in pregnancy suggests deficiency of:

A) Iron

B) Vitamin C

C) Vitamin D

D) Vitamin B12

Answer: A) Iron

23. Polyhydramnios in a diabetic mother usually indicates:

A) Good glycemic control

B) Poor glycemic control

C) Fetal lung maturity

D) Placental insufficiency

Answer: B) Poor glycemic control

24. Oligohydramnios with normal fetal kidneys may indicate:

A) Fetal anemia

B) Ruptured membranes

C) Maternal diabetes

D) Twin pregnancy

Answer: B) Ruptured membranes

25. In severe anemia near term, management may include:

A) Oral iron only

B) IV iron infusion

C) Blood transfusion

D) Both B and C

Answer: D) Both B and C

26. Polyhydramnios increases the risk of:

- A) Shoulder dystocia
- B) Cord prolapse
- C) Uterine rupture
- D) Placental abruption

Answer: B) Cord prolapse

27. Oligohydramnios can cause fetal:

- A) Pulmonary hypoplasia
- B) Macrosomia
- C) Hyperglycemia
- D) Polycythemia

Answer: A) Pulmonary hypoplasia

28. To assess fetal wellbeing in maternal anemia, use:

- A) Ultrasound for growth
- B) Maternal Hb only
- C) Fetal MRI
- D) Amniocentesis

Answer: A) Ultrasound for growth

29. In polyhydramnios, the fetal anomaly scan should focus on:

- A) Cardiac and renal systems
- B) Gastrointestinal and CNS
- C) Skeletal only
- D) Placental location only

Answer: B) Gastrointestinal and CNS

30. Severe oligohydramnios with fetal distress requires:

- A) Expectant management
 - B) Amnioinfusion
 - C) Maternal hydration
 - D) Immediate delivery
- Answer: D) Immediate delivery

2. Abnormal Labour, Malposition & Malpresentation (Q31 – 60)

31. The 3 Ps of abnormal labour refer to:

- A) Pain, Parity, Pelvis

- B) Power, Passenger, Passage
 - C) Presentation, Position, Progression
 - D) Pressure, Perfusion, Placenta
- Answer: B) Power, Passenger, Passage

32. Occiput posterior position commonly causes:

- A) Shoulder dystocia
- B) Severe maternal back pain
- C) Cord prolapse
- D) Uterine rupture

Answer: B) Severe maternal back pain

33. Deep transverse arrest occurs when the fetal head is stuck in:

- A) Occiput anterior
- B) Occiput posterior
- C) Occiput transverse
- D) Mentum anterior

Answer: C) Occiput transverse

34. A "passenger" cause of abnormal labour is:

- A) Hypotonic contractions
- B) Breech presentation
- C) Contracted pelvis
- D) Uterine fibroid

Answer: B) Breech presentation

35. Precipitate labour is defined as delivery within:

- A) 6 hours
- B) 4 hours
- C) 3 hours
- D) 2 hours

Answer: C) 3 hours

36. Brow presentation is typically managed by:

- A) Vaginal delivery
- B) External cephalic version
- C) Cesarean section
- D) Forceps rotation

Answer: C) Cesarean section

37. To assist rotation of an occiput posterior fetus, use:

- A) McRoberts position
- B) Hands-and-knees position
- C) Lithotomy position

D) Trendelenburg position

Answer: B) Hands-and-knees position

38. External cephalic version for breech is usually performed at:

A) 28 – 30 weeks

B) 32 – 34 weeks

C) 36 – 37 weeks

D) 38 – 40 weeks

Answer: C) 36 – 37 weeks

39. The most common type of breech presentation at term is:

A) Footling breech

B) Complete breech

C) Frank breech

D) Kneeling breech

Answer: C) Frank breech

40. Which pelvic shape predisposes to occiput posterior position?

A) Gynecoid

B) Android

C) Anthropoid

D) Platypelloid

Answer: B) Android

41. Hypertonic uterine contractions are managed by:

A) Increasing oxytocin

B) Stopping oxytocin

C) Amniotomy

D) Forceps delivery

Answer: B) Stopping oxytocin

42. Bandl's ring is a sign of:

A) Normal labour

B) Obstructed labour

C) Fetal distress

D) Maternal dehydration

Answer: B) Obstructed labour

43. The first step in managing shoulder dystocia is:

A) Fundal pressure

B) McRoberts manoeuvre

C) Immediate C-section

D) Symphysiotomy

Answer: B) McRoberts manoeuvre

44. In cord prolapse, immediate action includes:

- A) Pushing cord back in
- B) Elevating presenting part + emergency C-section
- C) Amnioinfusion
- D) Tocolysis

Answer: B) Elevating presenting part + emergency C-section

45. Prolonged second stage in a nulliparous woman is > ___ hours:

- A) 2
- B) 3
- C) 4
- D) 5

Answer: B) 3 hours

46. A maternal risk of malposition is:

- A) Placenta previa
- B) Polyhydramnios
- C) Perineal trauma
- D) Gestational diabetes

Answer: C) Perineal trauma

47. Face presentation can deliver vaginally only if:

- A) Mentum anterior
- B) Mentum posterior
- C) Brow presentation
- D) Occiput posterior

Answer: A) Mentum anterior

48. Hypotonic uterine inertia is managed with:

- A) Oxytocin augmentation
- B) Stopping contractions
- C) Emergency C-section
- D) Expectant management

Answer: A) Oxytocin augmentation

49. Compound presentation involves:

- A) Breech with extended legs
- B) Head and hand presenting together
- C) Twins both vertex
- D) Transverse lie with back down

Answer: B) Head and hand presenting together

50. Uterine tetany is often caused by:

- A) Insufficient oxytocin
- B) Oxytocin overdose
- C) Maternal dehydration
- D) Fetal macrosomia

Answer: B) Oxytocin overdose

51. Which is NOT a type of breech presentation?

- A) Frank breech
- B) Complete breech
- C) Footling breech
- D) Cephalic breech

Answer: D) Cephalic breech

52. In occiput posterior position, on vaginal exam you feel:

- A) Anterior fontanelle anteriorly
- B) Posterior fontanelle anteriorly
- C) Sagittal suture transverse
- D) No fontanelles palpable

Answer: A) Anterior fontanelle anteriorly

53. A risk factor for malpresentation is:

- A) Maternal age <20
- B) Normal BMI
- C) Polyhydramnios
- D) Primigravida

Answer: C) Polyhydramnios

54. If face presentation is mentum posterior:

- A) Vaginal delivery is easy
- B) Forceps can be used
- C) C-section is mandatory
- D) External version should be tried

Answer: C) C-section is mandatory

55. Deep transverse arrest requires:

- A) Expectant management
- B) Oxytocin augmentation only
- C) Manual rotation or C-section
- D) Vacuum extraction without rotation

Answer: C) Manual rotation or C-section

56. Precipitate labour can cause:

- A) Fetal intracranial hemorrhage
- B) Maternal hypertension

- C) Polyhydramnios
- D) Oligohydramnios

Answer: A) Fetal intracranial hemorrhage

57. Malpresentation increases risk of:

- A) Postpartum hemorrhage
- B) Cord prolapse
- C) Maternal diabetes
- D) Pre-eclampsia

Answer: B) Cord prolapse

58. In breech presentation, the highest risk for head entrapment is with:

- A) Frank breech
- B) Complete breech
- C) Footling breech
- D) All carry equal risk

Answer: C) Footling breech

59. To diagnose malposition, the best method is:

- A) Abdominal palpation
- B) Vaginal examination
- C) Ultrasound only
- D) CT scan

Answer: B) Vaginal examination

60. In breech presentation at 39 weeks, if external cephalic version fails:

- A) Induce labour immediately
- B) Attempt vaginal breech delivery
- C) Plan elective C-section
- D) Wait for spontaneous labour

Answer: C) Plan elective C-section

3. Prolonged Labour, Obstructed Labour, Maternal & Fetal Distress (Q61 – 90)

61. Prolonged latent phase is >20 hours in:

- A) Multiparous women
- B) Nulliparous women
- C) Both
- D) Neither

Answer: B) Nulliparous women

62. The most common cause of obstructed labour is:

- A) Uterine atony
- B) Cephalopelvic disproportion
- C) Cord prolapse
- D) Placenta previa

Answer: B) Cephalopelvic disproportion

63. Bandl's ring indicates:

- A) Normal labour progress
- B) Obstructed labour
- C) Fetal distress
- D) Maternal dehydration

Answer: B) Obstructed labour

64. Late decelerations on CTG suggest:

- A) Cord compression
- B) Uteroplacental insufficiency
- C) Fetal sleep
- D) Maternal fever

Answer: B) Uteroplacental insufficiency

65. Immediate action for fetal distress includes:

- A) Increase oxytocin
- B) Left lateral position
- C) Fundal pressure
- D) Vaginal exam

Answer: B) Left lateral position

66. Maternal distress is characterized by:

- A) Bradycardia
- B) Hypotension only
- C) Tachycardia and tachypnea
- D) Hypertension only

Answer: C) Tachycardia and tachypnea

67. Thick meconium suggests:

- A) Fetal hypoxia
- B) Normal variant
- C) Maternal infection
- D) Fetal sleep

Answer: A) Fetal hypoxia

68. Obstructed labour can lead to:

- A) Uterine rupture
- B) Placenta accreta